



Transesophageal Echocardiography Report

UNIT DIAGNOSTIK NON INVASIF
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TEE Procedure

Anesthesia : **Topical spray xylocain 10%** Sedation : **No** Duration : **15 min** Complication : **None**

Patient Information

Name : SARMINI SAIMAN	Study Date : 07/02/2024	Height : 165cm	Diagnosis : Post MVR + AVR mechanical valve
MRN : 2021488323	DOB : 16/11/1975	Weight : 65kg	Referring physician : dr. Rina Ariani, SpJP(K)
Age : 49	Gender : female	BSA : 1.72m²	Operator : RAR / GAT
First Write Time : 10/02/2024 08:36:34	Last Write Time : 13/02/2024 11:34:55	Blood Pressure : 118 / 67 mmHg	Medical Device : EPIQ CVx

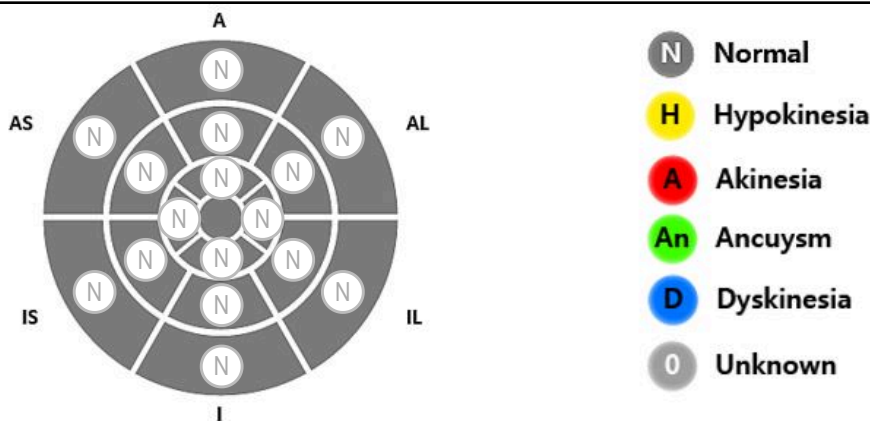
MMode / 2D Measurements & Calculations

IVSd : 12.6mm	Aor Diam : 30mm	EDV(teich) : 177ml	EF(Biplane) :
IVSs : 18.7mm	LA dimen : 49mm	ESV(teich) : 79ml	EF(4ch) : 64%
LVIDd : 59.5mm	LVOT diam : 18mm	EF : 55%	EF(2ch) :
LVIDs : 42.1mm	LA/Ao : 1.6mm	FS : 29.2%	IVC(Ins) :
LVPWd : 13.9mm	LAVI : 80ml/m²	TAPSE : 16mm	IVC(EXP) :
LVPWs : 18.2mm	LV mass Index : 206g/m²		

Doppler Measurements & Calculations

MV E vel :	E/E'(medial) :	MV V max : 1.8m/sec	MV VTI : 92.8cm
MV DT :	E/E'(lateral) :	MV mean PG : 6mmHg	MV PHT :
MV A vel :	E/E'(average) :	MVA(plan) : cm²	MVA(VTI) : 0.6cm²
E/A :			MVA by PHT :
AV V max : 2.8m/sec	AI PHT :	TR V max(systolic) : 2.4m/sec	
AV max PG : 31mmHg	AI(EROA) :	TR max PG(systolic) : 22mmHg	
AV mean PG : 17mmHg	LVOT VTI : 23cm	TV mean PG(diastolic) :	
AV VTI : 55.4cm	Desending Aorta(EDV) :	TV VTI :	
AVA (VTI) : 1.1cm²	SV(LVOT) : 58ml	PV Acc time : 137msec	
AVA(Vmax) : 1cm²	AVA(plan) : cm²		

2D Wall-Motion Diagram



Findings

ECG Rhythm	ECG is in atrial fibrillation.
Study Quality	The quality of the study is adequate.
Left Ventricle	Left ventricle is dilated. There is concentric left ventricular hypertrophy. RWT 0.46 ; LVMI 206 gr/m2. Global left ventricular systolic function is normal. LVEF 64% (Biplane).
Right Ventricle	The right ventricle is normal in size. Right ventricular systolic function is normal. TAPSE 16 mm ; RV FAC 59.8%.
Left Atrium	Left atrium is dilated. LAVI 80 mL/m2.
Right Atrium	Right atrium is dilated.
Interatrial Septum	Intact interatrial septum.
Interventricular septum	Intact interventricular septum.
Aortic Valve	Aortic valve post replace with mechanical valve. The opening prosthetic valve is not well visualized. The prosthetic valve is well-seated. No leakage is seen. Doppler evaluation showed borderline prosthetic aortic valve gradient. AV Vmax 3.03 m/s ; DVI 0.38 ; AV max PG 37 mmHg ; AV mean PG 22 mmHg ; Acceleration time 99 ms.EOAi 0.6 cm2/m2

Mitral Valve	Mitral valve post replace with mechanical valve. The prosthetic valve is well-seated. There is limited prosthetic valve opening with one disk completely immobile. There is possible thrombus on prosthetic valve. There is mild central leakage. Doppler evaluation showed increased prosthetic mitral valve gradient. Trans-prosthetic mitral gradient 8 mmHg.
Tricuspid Valve	There is moderate tricuspid regurgitation. TR VC 0.41 cm.
Pulmonic Valve	Pulmonic valve appears normal. There is no pulmonary regurgitation.
Mass/Thrombus	There is thrombus at LA and LAA.
Pericardium	Pericardium is normal.
Aorta	There is no atherome plaque seen at descending aorta.
Others	Low probability of pulmonary hypertension.

Conclusion

- Malfunction of mitral mechanical prosthetic valve, with limited opening of one leaflet possibly due to thrombus. Trans-prosthetic mitral gradient is increased.
- Post aortic mechanical prosthetic valve replacement. The opening prosthetic valve is not well visualized. Doppler evaluation showed borderline prosthetic aortic valve gradient suggestive PPM
- Moderate TR with low probability of PH
- Good LV systolic function, LVEF 64% (simpson)
- Global normokinetic
- Good RV contractility
- Thrombus is seen in LA and LAA with LASEC

Comments

Operator : RAR / GAT

Fellow : dr. Gusti Ayu Temi, SpJP

Supervisor Confirm : dr. Rina Ariani, Sp.JP

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Captured Patient Images

